

Appeal evaluation scenarios: the full range of outcomes

Every appeal that comes in is evaluated the same way — matched against the original denial, the member's benefit and eligibility information, their treatment history, and the health plan's clinical requirements. What differs from case to case is *which* of those checks the case passes or fails, and that's what determines the outcome. This reference walks through ten real scenarios — one approval, five different reasons an appeal can be rejected, and four other situations staff regularly encounter — each with a worked example.

Positive outcome

The appeal succeeds and the original denial is overturned

1. Approved — the missing evidence is provided

APPROVED

When this happens: the appeal supplies exactly the evidence the original denial said was missing, and every other requirement is already on file and met.

EXAMPLE
Eleanor Whitfield (case APL-2026-00312) appealed the denial of a heart device. The denial cited a lack of proof she'd completed the required medication trial first. Her treatment history on file showed the trial was in fact completed and documented — it simply hadn't been included in the original submission.

What determined the outcome: four of five clinical requirements were directly confirmed, and the fifth was undocumented rather than failed. Recommendation: approve, with one item flagged for the reviewer to confirm.

Rejection reasons

Five distinct reasons an appeal is denied — clinical, coverage, and procedural

2. Denied — clinical requirements genuinely not met

DENIED

When this happens: the case is reviewed in full, but the health plan's medical necessity requirements for the treatment simply aren't satisfied by anything in the documentation — not a paperwork gap, a genuine mismatch.

EXAMPLE
Robert Yeung (member MBR-330456) appealed a denial of weight-loss surgery. Plan requirements call for a body mass index of 40 or higher (or 35 with a related health condition), plus six months of physician-supervised weight management within the past two years. His records show a BMI of 34, no related condition, and only two months of supervised program participation.

What determined the outcome: the appeal restated the original request but did not add any new evidence, and the core numeric thresholds are not met. Recommendation: deny, original decision upheld.

3. Denied — the service isn't a covered benefit

DENIED

When this happens: the plan excludes the requested service outright, except under specific documented conditions — and those conditions aren't met, regardless of how reasonable the clinical case sounds.

EXAMPLE
Priya Anand (member MBR-118820) appealed a denial of eyelid surgery. Her plan excludes this procedure as cosmetic, unless a visual field test shows at least 30% obstruction. No visual field test was included anywhere in her records.

What determined the outcome: the plan's exclusion applies unless a specific test result is on file, and it isn't. Recommendation: deny — the appeal notes that resubmission with visual field test results, if the obstruction is present, could change the outcome.

4. Denied — annual limit exceeded, no override provided

DENIED

When this happens: the request would exceed a hard yearly limit on a benefit, and the plan's specific requirements for approving an exception aren't addressed in the appeal at all — this is a harder denial than a documentation gap, because nothing was submitted to work with.

EXAMPLE
David Okafor (member MBR-772341) has used all 20 of his annual physical therapy visits for knee pain and is requesting 15 more. The appeal states only that "the patient is still in pain" — no functional test score, no specific goal, and no estimate of how many additional visits would be needed.

What determined the outcome: the plan allows exceptions to the annual limit, but only with a specific package of evidence, none of which was provided. Recommendation: deny, with a note describing exactly what would need to be submitted to reconsider.

5. Denied — member wasn't covered on the date of service

DENIED

When this happens: the member's coverage had lapsed — often for non-payment — on the specific date the service was provided, regardless of whether the service itself was medically appropriate.

EXAMPLE
Helena Brooks (member MBR-904433) appealed a denied MRI performed on July 2, 2026. Her plan's enrollment record shows coverage lapsed on June 15, 2026 and wasn't reinstated until August 1, 2026 — the service date falls squarely in that gap.

What determined the outcome: this is an eligibility issue, not a clinical one — no amount of medical documentation changes whether the member was covered that day. Recommendation: deny; member may separately dispute the coverage lapse itself if she believes it's inaccurate.

6. Denied — appeal filed after the deadline

DENIED

When this happens: health plans set a window for filing an appeal after a denial — commonly 180 days — and the appeal arrives after that window has closed, with no explanation for the delay.

EXAMPLE
Thomas Reyes (member MBR-661209) was denied on November 2, 2025, giving him until May 1, 2026 to appeal. His appeal was received on July 14, 2026 — 74 days after the deadline, with no reason given for the delay.

What determined the outcome: this is a procedural denial, separate from the clinical merits of the request, which were never reached. Recommendation: deny as untimely; the member retains the option to request a good-cause exception if he can provide a valid reason for the delay.

Held pending more information

The case is close, but one specific piece of evidence is missing

7. Needs more information — a specific documentation gap

NEEDS INFO

When this happens: most of what's required is present and the appeal shows real progress toward meeting the requirements, but one specific, identifiable piece of evidence is missing — not a wholesale mismatch, just a gap.

EXAMPLE
Marcus Oyelaran appealed a denial of 12 additional physical therapy visits after back surgery. His functional test score improved meaningfully over eight weeks — good evidence — but the plan also requires a specific stated goal and an estimated number of visits needed to reach it, and neither was included.

What determined the outcome: one of the plan's required elements for a visit extension is simply absent, while the others are well documented. Recommendation: hold and request the missing goal statement from the treating therapist, rather than deny outright.

Cannot be evaluated yet

There isn't enough information to identify the member at all

8. Cannot process — not enough information to identify the member

ON HOLD

When this happens: the appeal simply doesn't contain enough information — no member ID, no date of birth, sometimes not even the full name — to safely match it to a record. Guessing based on a first name and a general description is not an acceptable substitute.

EXAMPLE
An email from Denise Carter asks for help with "my mom's physical therapy" that was denied. She gives her mother's first name and mentions a hip surgery, but no member ID, date of birth, or case number — nowhere near enough to confirm which member's record this actually is.

What determined the outcome: even though a matching record likely exists somewhere in the system, proceeding without confirming identity risks acting on the wrong member's information. Recommendation: hold and request the member ID or date of birth before anything else happens.

Special handling

Situations that aren't really approve/deny decisions at all

9. Duplicate — the same appeal arrives twice

MERGED

When this happens: a new email or upload references a case number that already has an open appeal in progress — often the same office resending the same request, sometimes out of concern the first submission wasn't received.

EXAMPLE

Three days after Eleanor Whitfield's original appeal (case APL-2026-00312) was received, a near-identical email arrives from the same cardiology office, referencing the same case number and the same request.

What determined the outcome: this isn't a new case — it's the same one. Recommendation: merge into the existing case rather than opening a second file or starting a second evaluation from scratch.

10. Escalated — a second appeal on a decision already upheld

ESCALATED

When this happens: a member or provider disputes a denial that has already been through one internal appeal and was upheld — most health plans allow only one internal appeal per denial before further disputes go to an outside reviewer.

EXAMPLE

Angela Ruiz (member MBR-556012) had an appeal reviewed and upheld on July 10, 2026. A new message disputing the same denial arrives on August 25, 2026, making largely the same argument as the first appeal.

What determined the outcome: policy limits members to one internal appeal per denial. Recommendation: route to an independent external review rather than repeating the internal evaluation a second time.

Quick reference

Scenario	Outcome	In one line
1. Missing evidence supplied	Approved	The exact gap the denial cited turns out to be documented after all.
2. Clinical requirements not met	Denied	Thresholds genuinely aren't met, and the appeal adds no new evidence.
3. Not a covered benefit	Denied	Plan excludes the service unless a specific condition is documented — it isn't.
4. Annual limit exceeded, no override	Denied	Exceeds a hard cap with none of the required override evidence submitted.
5. Not eligible on date of service	Denied	Coverage had lapsed on the day the service was provided.
6. Filed after the deadline	Denied	Appeal arrived after the filing window closed, with no reason given.
7. Specific documentation gap	Needs info	Most requirements met; one identifiable item is missing.
8. Member can't be identified	On hold	Not enough information to safely match a member record.
9. Same appeal received twice	Merged	Duplicate submission of an appeal already in progress.
10. Prior internal appeal already upheld	Escalated	Routed to external review rather than a repeat internal look.

These ten cover the situations appeals staff encounter most often, built from real examples rather than hypotheticals. Any appeal that doesn't clearly fit one of these patterns should be routed to a senior reviewer rather than forced into the nearest category.